

24-Jan-2025

SUSPECTED UNEXPECTED SERIOUS ADVERSE REACTION (SUSAR) REPORT

SUSAR Report for Nivolumab (BMS-936558), # 2024-191312

Reference Document: Investigator brochure, Version 23 (27-Jun-2024)

Protocol: XL092-002

Dear Investigator:

As the sponsor of clinical studies with the investigational medicinal product (IMP) specified above, Bristol-Myers Squibb Research & Development is issuing this safety report. The details of this adverse event/safety issue are being reported to you in compliance with regulations.

For Suspected Unexpected Serious Adverse Reactions (SUSARs) occurring in blinded clinical trials, study treatments will remain blinded in reports sent to investigators.

The attached report does not reflect a conclusion or constitute an admission by the sponsor that the IMP caused or contributed to this adverse event/safety issue.

Your Institutional Review Board (IRB)/Ethics Committee (EC) must receive a copy of this SUSAR as per applicable regulation and local IRB/EC policy. Please consult the clinical trial personnel or documents/manuals regarding your responsibility for submitting SUSARs to your IRB/EC. Please also review your IRB/EC procedures relating to adverse events/safety issues that may necessitate changes to the Informed Consent Form.

Please include this SUSAR with all copies of the Reference Document.

We thank you in advance for your prompt attention to this information.

Karthik Muthusami

Head, Expedited Safety Reporting

Worldwide Patient Safety

Attachment: CIOMS Report

SUSPECT ADVERSE REACTION REPORT 2024-191312(2)												

I. REACTION INFORMATION

1. PATIENT INITIALS (first, last) [REDACTED]	1a. COUNTRY UNITED Cont..	2. DATE OF BIRTH Day Month Year [REDACTED] [REDACTED] [REDACTED]	2a. AGE Years 71	3. SEX Male	4-6 REACTION ONSET Day Month Year 02 Dec 2024	8-12 CHECK ALL APPROPRIATE TO ADVERSE REACTION ☒ PATIENT DIED ☒ LIFE THREATENING ☒ INVOLVED OR PROLONGED INPATIENT HOSPITALIZATION ☒ RESULTS IN PERSISTENCE OR SIGNIFICANT DISABILITY/INCAPACITY ☐ CONGENITAL ANOMALY ☒ OTHER MEDICALLY IMPORTANT CONDITION
7+13 DESCRIBE REACTION(S) (including relevant tests/lab data) MedDRA Version : v.27.1 1) PROGRESSION OF DISEASE UNDER STUDY (MCRC) (Malignant neoplasm progression (10051398), Malignant neoplasm progression (10051398)) (12/Dec/2024 -) - Fatal 2) HYPOTHERMIA (Hypothermia (10021113), Hypothermia (10021113)) (03/Dec/2024 -) - Recovering/Resolving 3) BILATERAL UPPER EXTREMITY WEAKNESS (R>L), R FOOT WEAKNESS (Weakness in extremity (10065702), Muscular weakness (10028372)) (02/Dec/2024 -) - Recovering/Resolving 4) PLEURAL EFFUSION (Pleural effusion (10035598), Pleural effusion (10035598)) (02/Dec/2024 -) - Recovering/Resolving Cont..						

II. SUSPECT DRUG(S) INFORMATION

14. SUSPECT DRUG(S)(include generic name) 1) NIVOLUMAB (NIVOLUMAB) (Suspect) (Verum) (Solution for injection)(Unknown) Cont..		20. DID EVENT ABATE AFTER STOPPING DRUG? ☐ YES ☐ NO ☒ NA 21. DID EVENT REAPPEAR AFTER REINTRODUCTION ☐ YES ☐ NO ☒ NA (NA : Not Applicable)
15. DAILY DOSE(S) 1) 22.85 milligram(s) (480 milligram(s), 1 in 3 Week)	16. ROUTE(S) OF ADMINISTRATION 1) Intravenous (not otherwise specified)	
17. INDICATION(S) FOR USE 1) Colorectal cancer [10061451 - Colorectal cancer] Cont..		
18. THERAPY DATE(S) (from/to) 1) (25/Mar/2024 - 04/Nov/2024)	19. THERAPY DURATION 1) 225 Days	

III. CONCOMITANT DRUG(S) AND HISTORY

22. CONCOMITANT DRUG(S) AND DATES OF ADMINISTRATION (exclude those used to treat reaction) 1) ATIVAN(LORAZEPAM)(06/Mar/2024 - Unknown) Cont..
23. OTHER RELEVANT HISTORY (e.g. diagnostics, allergies, pregnancy with last month of period, etc.) MedDRA Version : v.27.1 1) TYPE 2 DIABETES MELLITUS (10067585, Type 2 diabetes mellitus) (23/Nov/2016 - Unknown) (Continuing: Unknown) Cont..

IV. MANUFACTURER INFORMATION

24a. NAME AND ADDRESS OF MANUFACTURER Name : BMS UNITED STATES OF AMERICA aepbusinessprocess@bms.com		Study Information Study Name: A Dose-Escalation and Expansion St (Cont..) EudraCT Number: Unknown Protocol No.: XL092-002 Center No.: Unknown Subject Id : 12301597
24. REPORT NULLIFIED ☐ YES ☐ NO	24b. MFR CONTROL NO. 2024-191312(2)	
24c. DATE RECEIVED BY MANUFACTURER 14/Jan/2025	24d. REPORT SOURCE ☒ STUDY ☐ LITERATURE ☒ HEALTH PROFESSIONAL	
DATE OF THIS REPORT 23/Jan/2025	25a. REPORT TYPE ☐ INITIAL ☒ FOLLOWUP	

= Continuation attached sheet(s)..

Continuation Sheet for CIOMS report

1a. COUNTRY

UNITED STATES OF AMERICA

7+13 DESCRIBE REACTION(S) (including relevant tests/lab data) (Continuation...)

Event Description :

Protocol title (XL092-002/CA209-6KE): A Dose-Escalation and Expansion Study of the Safety and Efficacy of XL092 in Combination with Immuno-Oncology Agents in Subjects with Unresectable Advanced or Metastatic Solid Tumors

PID: XL092-002 (CA209-6KE)-Unk-1230-1597/ 2024-191312/ United States of America

Reportable SAE-Downgrade: Fatal Malignant Neoplasm Progression (Verbatim: Progression of Disease Under Study [MCRC]).

Other SAEs: Life-threatening (LT) Hypothermia, Muscular Weakness (Verbatim: Bilateral Upper Extremity Weakness (R&L), R Foot Weakness), and Pleural Effusion

A clinical investigator reported that a 71-year-old male patient was hospitalized due to CTC grade 3 events of bilateral upper extremity weakness (with concern for metastasis to the spine with subsequent cord involvement), which led to disability, and pleural effusion approximately eight months after the initiation of study therapy with XL092 and nivolumab for colorectal cancer. The following day, he was noted to have CTC grade 2 LT hypothermia, which prolonged his hospitalization. Approximately one week later, the patient died due to the progression of metastatic colorectal cancer. XL092 and nivolumab were discontinued due to radiographic disease progression prior to the onset of these events.

The patient's medical history included gastroesophageal reflux disease (GERD), type II diabetes mellitus, dry skin, sensory peripheral neuropathy, leukopenia, sinus tachycardia, chronic kidney disease, and hypertension (x2). Relevant procedure history included flexible sigmoidoscopy, colonoscopy with biopsy, and colonoscopy with biopsy. Relevant surgical history included larynx surgery, hepatic resection, subsegtomy and microwave ablation of segment 5 and cholecystectomy, right hepatic sectionectomy, segmentectomy 6/7, repair ivc and diaphragm.

Concomitant medications included lorazepam, atorvastatin, glipizide, hydrochlorothiazide, loperamide, lidocaine and prilocaine, atropine sulfate, losartan, magnesium oxide, metformin hydrochloride, multivitamin, nifedipine, mirtazapine, and ondansetron.

Past treatment included fluorouracil (x4), oxaliplatin (x3), capecitabine (x5), irinotecan (x2), panitumumab (x2), and bevacizumab (x5), leucovorin(x3) for colorectal cancer.

On 25-Mar-2024, open-label study therapy was initiated with intravenous (IV) nivolumab (480 mg; every three weeks [Q3WK]) and oral (PO) XL092 (100 mg; daily [QD]). Prior to the onset of the events, the patient received the most recent dose of XL092 on 03-Nov-2024. On 04-Nov-2024, XL092 and nivolumab were discontinued due to the radiographic progression.

On 02-Dec-2024, the patient was admitted to the hospital due to the bilateral upper extremity weakness (right and left) and right foot weakness with concern for metastasis to the spine with subsequent cord involvement. Extremity pain/weakness began two weeks before and had been getting progressively worse. The patient was evaluated for a stroke, but investigations were negative. According to the radiology resident, there was cervical spine (CF) (C3-C4) compression but the final read of the magnetic resonance imaging (MRI) of the cervical, thoracic, and lumbar spine (C/T/L spine) was pending. Throughout the night, the patient was noted to have progressive hypothermia (at 15:59, 35.3 C; at 18:30, 35.6 C; and between 95.5 Degrees Fahrenheit and 94.4 Degrees Fahrenheit) despite conservative heating measures and the initiation of antibiotics. The patient did not have infectious symptoms on admission, but on the same day, chest x-ray showed moderate right-sided pleural effusion with commiserate right middle and lower lobe atelectasis. It was noted hospitalization was prolonged due to pleural effusion.

On 03-Dec-2024, computed tomography (CT) of entire spine routine without contrast showed numerous pulmonary nodules redemonstrated (reflective of metastatic spread), which placed him at risk for an infective process in the lung. An infectious workup was initiated, and he was started on empiric ceftriaxone (CTX). No clinical evidence of infection was found, given the lack of fever, chills, cough, or elevated white blood cell count. Most likely, he had a malignant pleural effusion. On the same day, the patient had hypothermia and hospitalization was prolonged due to it. Body temperature was recorded as follows: at 01:00, 34.9 C; at 01:51, 34.9 C; at 01:52, 94.4 F; at 03:29, 92.2 F; at 04:21, 34.6 C; at 05:00, 34.9 C; at 05:52, 35.6 C; and at 08:00, 35.8 C. The patient had also experienced acute hypoxic respiratory failure, likely secondary to moderate pleural effusion with right middle lobe (RML) and right middle lobe atelectasis. He had experienced intermittent desaturations to the high 80s. The patient also presented with hypotension, lactic acidosis, and metabolic acidosis. Given poor oral intake over weeks to months, it was suspected that the patient was volume depleted.

On 12-Dec-2024, the patient died due to progression of metastatic colorectal cancer (MCRC). An autopsy was not performed. No other information was reported. At the time of the patient's death, of the hypothermia, pleural effusion, bilateral upper extremity weakness and R foot weakness were considered resolving.

The investigator assessed pleural effusion, bilateral upper extremity weakness (R&L) and R foot weakness, and progression of disease under study (MCRC) as related to underlying condition of colorectal cancer.

Supplemental information received on 14-Jan-2025 from the clinical investigator updated the causality for the event progression of disease under study (MCRC) as not related to XL092 and nivolumab study therapy (previously causality was not provided). Based on this information, this case no longer meets expedited safety reporting criteria. All information has been updated into the narrative.

Continuation Sheet for CIOMS report

Investigator causality assessment: Fatal progression of disease under study (MCRC), LT hypothermia, pleural effusion, bilateral upper extremity weakness (R&L) and R foot weakness were not related to XL092 and nivolumab.

BMS causality assessment: Fatal progression of disease under study (MCRC), LT hypothermia, pleural effusion, bilateral upper extremity weakness (R&L) and R foot weakness were not related to XL092 and nivolumab.

Expectedness Assessment:

Nivolumab (IB): Fatal malignant neoplasm progression, LT hypothermia, muscular weakness and pleural effusion were considered unlisted.

Company Remarks (Sender's Comments) :

This elderly patient was hospitalized due to events of bilateral upper extremity weakness (with concern for metastasis to the spine with subsequent cord involvement), and pleural effusion approximately eight months after the initiation of study therapy with XL092 and nivolumab for colorectal cancer. The following day, he had hypothermia, which was life-threatening and prolonged his hospitalization. Approximately one week later, the patient died due to the progression of metastatic colorectal cancer. Radiology consultation reported cervical spine (C3-C4) compression. Computed tomography showed numerous pulmonary nodules suggestive of metastatic spread. Underlying progressive malignancy likely led to bilateral upper extremity weakness and pleural effusion. Cervical spinal cord compression and elderly age could have contributed to hypothermia. Considering the inherent nature of progression of underlying cancer along with a fatal outcome, these events are assessed as not related to the study therapy.

Additional Information (Continuation...)

Laboratory Data :

02 Dec 2024: CHEST X-RAY: MODERATE RIGHT-SIDED PLEURAL EFFUSION WITH COMMISERATE RIGHT MIDDLE AND LOWER LOBE ATELECTASIS

02 Dec 2024: TEMPERATURE: 15:59, 35.3 C; at 18:30, 35.6 C; and between 95.5°F and 94.4°F

03 Dec 2024: TEMPERATURE: 01:00, 34.9 C; at 01:51, 34.9 C; at 01:52, 94.4 F; at 03:29, 92.2 F; at 04:21, 34.6 C; at 05:00, 34.9 C; at 05:52, 35.6 C; and at 08:00, 35.8 C.

03 Dec 2024: CT ENTIRE SPINE ROUTINE WITHOUT CONTRAS: NUMEROUS PULMONARY NODULES REDEMONSTRATED, BETTER EVALUATED ON RECENT CHEST CT See Event Description for additional lab results

Lab Result :

Test Name	Test Date	Test Result	Normal Value
BIOPSY	15/Sep/2017		
BODY TEMPERATURE	02/Dec/2024		
BODY TEMPERATURE	02/Dec/2024 15:59:00	35.3 degree Celsius	
BODY TEMPERATURE	02/Dec/2024 18:30:00	35.6 degree Celsius	
BODY TEMPERATURE	03/Dec/2024 01:00:00	34.9 degree Celsius	
BODY TEMPERATURE	03/Dec/2024 01:51:00	34.9 degree Celsius	
BODY TEMPERATURE	03/Dec/2024 01:52:00	94.4 degree Fahrenheit	
BODY TEMPERATURE	03/Dec/2024 03:29:00	92.2 degree Fahrenheit	
BODY TEMPERATURE	03/Dec/2024 04:21:00	34.6 degree Celsius	
BODY TEMPERATURE	03/Dec/2024 05:00:00	34.9 degree Celsius	
BODY TEMPERATURE	03/Dec/2024 05:52:00	35.6 degree Celsius	
BODY TEMPERATURE	03/Dec/2024 08:00:00	35.8 degree Celsius	

Continuation Sheet for CIOMS report

CHEST X-RAY	02/Dec/2024		
COLONOSCOPY	15/Sep/2017		
COMPUTED TOMOGRAPHY (CT) OF ENTIRE SPINE WITHOUT CONTRAST	03/Dec/2024		
INFECTIOUS WORKUP	03/Dec/2024		
INVESTIGATION NOS	02/Dec/2024		
MAGNETIC RESONANCE IMAGING (MRI) OF CERVICAL	02/Dec/2024		
MAGNETIC RESONANCE IMAGING (MRI) OF LUMBAR SPINE (C/T/L SPINE	02/Dec/2024		
MAGNETIC RESONANCE IMAGING (MRI) OF THORACIC	02/Dec/2024		
SIGMOIDOSCOPY	02/Aug/2019		
WORKUP FOR STROKE	02/Dec/2024		

Test Result (Code) / Result Unstructured Data (free text) :

1) Test Name: BIOPSY

Result Unstructured Data (free text) : no results provided

Test Date: 15/Sep/2017

2) Test Name: BODY TEMPERATURE

Result Unstructured Data (free text) : between 95.5°F and 94.4°F

Test Date: 02/Dec/2024

13) Test Name: CHEST X-RAY

Result Unstructured Data (free text) : chest x-ray performed which showed moderate right-sided pleural effusion with commiserate right middle and lower lobe atelectasis

Test Date: 02/Dec/2024

14) Test Name: COLONOSCOPY

Result Unstructured Data (free text) : no results provided

Test Date: 15/Sep/2017

15) Test Name: COMPUTED TOMOGRAPHY (CT) OF ENTIRE SPINE WITHOUT CONTRAST

Result Unstructured Data (free text) : showed numerous pulmonary nodules redemonstrated (reflective of metastatic spread), which placed him at risk for an infective process in the lung

Test Date: 03/Dec/2024

16) Test Name: INFECTIOUS WORKUP

Result Unstructured Data (free text) : No clinical evidence of infection was found, given the lack of fever, chills, cough, or elevated white blood cell count.

Test Date: 03/Dec/2024

17) Test Name: INVESTIGATION NOS

Result Unstructured Data (free text) : radiology resident, there was cervical spine (C3-C4) compression

Test Date: 02/Dec/2024

18) Test Name: MAGNETIC RESONANCE IMAGING (MRI) OF CERVICAL

Result Unstructured Data (free text) : results not provided

Test Date: 02/Dec/2024

19) Test Name: MAGNETIC RESONANCE IMAGING (MRI) OF LUMBAR SPINE (C/T/L SPINE

Result Unstructured Data (free text) : results not provided

Test Date: 02/Dec/2024

20) Test Name: MAGNETIC RESONANCE IMAGING (MRI) OF THORACIC

Continuation Sheet for CIOMS report

Result Unstructured Data (free text) : results not provided

Test Date: 02/Dec/2024

21) Test Name: SIGMOIDOSCOPY

Result Unstructured Data (free text) : no results provided

Test Date: 02/Aug/2019

22) Test Name: WORKUP FOR STROKE

Result Unstructured Data (free text) : the workup was negative

Test Date: 02/Dec/2024

14.SUSPECT DRUG(S) (Continuation...)

Product-Reaction Level

1) Drug : NIVOLUMAB
 Active Substance : 1) NIVOLUMAB
 Coding Class : Verum
 Drug Characterization : Suspect
 Form of Admin : 1) Solution for injection
 Lot Number : 1) Unknown
 Daily Dose : 1) 22.85 milligram(s) (480 milligram(s), 1 in 3 Week)
 Route of Admin : 1) Intravenous (not otherwise specified)
 Indications : 1) Colorectal cancer [10061451 - Colorectal cancer]
 Therapy Dates : 1) From : 25/Mar/2024 To :04/Nov/2024
 Therapy Duration : 1) 225 Days
 Action(s) Taken With Drug : Not applicable

Causality

1) PROGRESSION OF DISEASE UNDER STUDY (MCRC) (Malignant neoplasm progression - 10051398, Malignant neoplasm progression - 10051398)

Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable

2) HYPOTHERMIA (Hypothermia - 10021113, Hypothermia - 10021113)

Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable

3) BILATERAL UPPER EXTREMITY WEAKNESS (R>L), R FOOT WEAKNESS (Weakness in extremity - 10065702, Muscular weakness - 10028372)

Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable

4) PLEURAL EFFUSION (Pleural effusion - 10035598, Pleural effusion - 10035598)

Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable

Labeling :

1) PROGRESSION OF DISEASE UNDER STUDY (MCRC)

CORE Labeled
 IB UnLabeled
 UNITED STATES OF AMERICA Labeled

2) HYPOTHERMIA

CORE UnLabeled
 IB UnLabeled
 UNITED STATES OF AMERICA UnLabeled

3) BILATERAL UPPER EXTREMITY WEAKNESS (R>L), R FOOT WEAKNESS

CORE UnLabeled
 IB UnLabeled
 UNITED STATES OF AMERICA UnLabeled

4) PLEURAL EFFUSION

CORE Labeled
 IB UnLabeled

Continuation Sheet for CIOMS report

UNITED STATES OF AMERICA

UnLabeled

2) Drug : XL092
 Active Substance : 1) XL092
 Coding Class : Verum
 Drug Characterization : Suspect
 Form of Admin : 1) Tablet
 Lot Number : 1) Unknown
 Daily Dose : 1) 100.0 milligram(s) (100 milligram(s), 1 in 1 Day)
 Route of Admin : 1) Oral
 Indications : 1) Colorectal cancer [10061451 - Colorectal cancer]
 Therapy Dates : 1) From : 25/Mar/2024 To :04/Nov/2024
 Therapy Duration : 1) 225 Days
 Action(s) Taken With Drug : Not applicable

Causality

1) PROGRESSION OF DISEASE UNDER STUDY (MCRC) (Malignant neoplasm progression - 10051398, Malignant neoplasm progression - 10051398)

Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable

2) HYPOTHERMIA (Hypothermia - 10021113, Hypothermia - 10021113)

Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable

3) BILATERAL UPPER EXTREMITY WEAKNESS (R>L), R FOOT WEAKNESS (Weakness in extremity - 10065702, Muscular weakness - 10028372)

Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable

4) PLEURAL EFFUSION (Pleural effusion - 10035598, Pleural effusion - 10035598)

Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable

3) Drug : RELATLIMAB,NIVOLUMAB_JPN

Active Substance : 1) RELATLIMAB
 2) NIVOLUMAB
 Drug Characterization : Drug Not Administered
 Form of Admin : 1) Solution for injection
 Lot Number : 1) Unknown
 Daily Dose : 1) 22.85 milligram(s) (480 milligram(s), 1 in 3 Week)
 Route of Admin : 1) Intravenous (not otherwise specified)
 Indications : 1) colorectal cancer [10061451 - Colorectal cancer]
 Therapy Dates :
 Therapy Duration :
 Action(s) Taken With Drug : Not applicable

Causality

1) PROGRESSION OF DISEASE UNDER STUDY (MCRC) (Malignant neoplasm progression - 10051398, Malignant neoplasm progression - 10051398)

Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable

2) HYPOTHERMIA (Hypothermia - 10021113, Hypothermia - 10021113)

Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable

3) BILATERAL UPPER EXTREMITY WEAKNESS (R>L), R FOOT WEAKNESS (Weakness in extremity - 10065702, Muscular weakness - 10028372)

Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable

Continuation Sheet for CIOMS report

ReChallenge : Not Applicable
 4) PLEURAL EFFUSION (Pleural effusion - 10035598, Pleural effusion - 10035598)
 Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable

Labeling :

1) PROGRESSION OF DISEASE UNDER STUDY (MCRC)
 IB UnLabeled
 2) HYPOTHERMIA
 IB UnLabeled
 3) BILATERAL UPPER EXTREMITY WEAKNESS (R>L), R FOOT WEAKNESS
 IB UnLabeled
 4) PLEURAL EFFUSION
 IB UnLabeled

4) Drug : BMS-986489-01_JPN
 Active Substance : 1) NIVOLUMAB
 2) BMS-986012-01
 Drug Characterization : Drug Not Administered
 Form of Admin : 1) Solution for injection
 Lot Number : 1) Unknown
 Daily Dose : 1) 22.85 milligram(s) (480 milligram(s), 1 in 3 Week)
 Route of Admin : 1) Intravenous (not otherwise specified)
 Indications : 1) COLORECTAL CANCER [10061451 - Colorectal cancer]
 Therapy Dates : 1) From : 25/Mar/2024 To :04/Nov/2024
 Therapy Duration : 1) 225 Days
 Action(s) Taken With Drug : Not applicable

Causality

1) PROGRESSION OF DISEASE UNDER STUDY (MCRC) (Malignant neoplasm progression - 10051398, Malignant neoplasm progression - 10051398)
 Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable
 2) HYPOTHERMIA (Hypothermia - 10021113, Hypothermia - 10021113)
 Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable
 3) BILATERAL UPPER EXTREMITY WEAKNESS (R>L), R FOOT WEAKNESS (Weakness in extremity - 10065702, Muscular weakness - 10028372)
 Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable
 4) PLEURAL EFFUSION (Pleural effusion - 10035598, Pleural effusion - 10035598)
 Causality as per reporter : Not Related
 Causality as per Mfr : Not Related
 DeChallenge : Not applicable
 ReChallenge : Not Applicable

Labeling :

1) PROGRESSION OF DISEASE UNDER STUDY (MCRC)
 IB UnLabeled
 2) HYPOTHERMIA
 IB UnLabeled
 3) BILATERAL UPPER EXTREMITY WEAKNESS (R>L), R FOOT WEAKNESS
 IB UnLabeled
 4) PLEURAL EFFUSION
 IB UnLabeled

15. DAILY DOSE(S) (Continuation...)

Dosage Text :

Drug 4 :BMS-986489-01_JPN

Continuation Sheet for CIOMS report

1) UNK

22.CONCOMITANT DRUG(S) (Continuation...)

1). Drug	:	ATIVAN
Active Substance	:	1) LORAZEPAM
Form Strength	:	
Form of Admin	:	1) Unknown
Route of Admin	:	1) Unknown
Indications	:	1) Anxiety [10002855 - Anxiety]
Therapy Dates	:	1) From : 06/Mar/2024 To : Unknown
Dosage Text	:	1) UNK
2). Drug	:	ATORVASTATIN
Active Substance	:	1) ATORVASTATIN
Form Strength	:	
Form of Admin	:	1) Unknown
Route of Admin	:	1) Unknown
Indications	:	1) Hypertension [10020772 - Hypertension]
Therapy Dates	:	1) From : 20/Feb/2017 To : Unknown
Dosage Text	:	1) UNK
3). Drug	:	Glipizide
Active Substance	:	1) GLIPIZIDE
Form Strength	:	
Form of Admin	:	1) Unknown
Route of Admin	:	1) Unknown
Indications	:	1) Type 2 diabetes mellitus [10067585 - Type 2 diabetes mellitus]
Therapy Dates	:	1) From : 23/Nov/2016 To : Unknown
Dosage Text	:	1) UNK
4). Drug	:	HYDROCHLOROTHIAZIDE
Active Substance	:	1) HYDROCHLOROTHIAZIDE
Form Strength	:	
Form of Admin	:	1) Unknown
Route of Admin	:	1) Unknown
Indications	:	1) Hypertension [10020772 - Hypertension]
Therapy Dates	:	1) From : 04/Apr/2023 To : Unknown
Dosage Text	:	1) UNK
5). Drug	:	IMODIUM
Active Substance	:	1) LOPERAMIDE HYDROCHLORIDE
Form Strength	:	
Form of Admin	:	1) Unknown
Route of Admin	:	1) Unknown
Indications	:	1) Diarrhoea [10012735 - Diarrhoea]
Therapy Dates	:	1) From : 03/Apr/2024 To : Unknown
Dosage Text	:	1) UNK
6). Drug	:	EMLA
Active Substance	:	1) LIDOCAINE
	:	2) PRILOCAINE
Form Strength	:	
Form of Admin	:	1) Unknown
Route of Admin	:	1) Unknown
Indications	:	1) Pain prophylaxis [10033484 - Pain prophylaxis]
Therapy Dates	:	1) From : 14/Sep/2022 To : Unknown
Dosage Text	:	1) UNK
7). Drug	:	LOMOTIL [ATROPINE SULFATE;DIPHENOXYLATE HYDROCHLORIDE]

Continuation Sheet for CIOMS report

Active Substance	:	1) ATROPINE SULFATE 2) DIPHENOXYLATE HYDROCHLORIDE
Form Strength	:	
Form of Admin	:	1) Unknown
Route of Admin	:	1) Unknown
Indications	:	1) Diarrhoea [10012735 - Diarrhoea]
Therapy Dates	:	1) From : 05/Aug/2024 To : Unknown
Dosage Text	:	1) UNK
8). Drug	:	LOSARTAN
Active Substance	:	1) LOSARTAN
Form Strength	:	
Form of Admin	:	1) Unknown
Route of Admin	:	1) Unknown
Indications	:	1) Hypertension [10020772 - Hypertension]
Therapy Dates	:	1) From : 20/Feb/2017 To : Unknown
Dosage Text	:	1) UNK
9). Drug	:	MAG-OX
Active Substance	:	1) MAGNESIUM OXIDE
Form Strength	:	
Form of Admin	:	1) Unknown
Route of Admin	:	1) Unknown
Indications	:	1) Hypertension [10020772 - Hypertension]
Therapy Dates	:	1) From : 18/Oct/2021 To : Unknown
Dosage Text	:	1) UNK
10). Drug	:	METFORMIN HCL
Active Substance	:	1) METFORMIN HYDROCHLORIDE
Form Strength	:	
Form of Admin	:	1) Unknown
Route of Admin	:	1) Unknown
Indications	:	1) Gestational diabetes [10018209 - Gestational diabetes]
Therapy Dates	:	1) From : 11/May/2017 To : Unknown
Dosage Text	:	1) UNK
11). Drug	:	MULTIVITAMIN [VITAMINS NOS]
Active Substance	:	1) VITAMINS NOS
Form Strength	:	
Form of Admin	:	1) Unknown
Route of Admin	:	1) Unknown
Indications	:	1) Supplementation therapy [10066215 - Supplementation therapy]
Therapy Dates	:	1) From : 16/Nov/2022 To : Unknown
Dosage Text	:	1) UNK
12). Drug	:	NIFEDIPINE
Active Substance	:	1) NIFEDIPINE
Form Strength	:	
Form of Admin	:	1) unknow
Route of Admin	:	1) Unknown
Indications	:	1) Hypertension [10020772 - Hypertension]
Therapy Dates	:	1) From : 19/Dec/2017 To : Unknown
Dosage Text	:	1) UNK
13). Drug	:	REMERON
Active Substance	:	1) MIRTAZAPINE
Form Strength	:	
Form of Admin	:	1) Unknown
Route of Admin	:	1) Unknown
Indications	:	1) Decreased appetite [10061428 - Decreased appetite]
Therapy Dates	:	1) From : 12/Aug/2024 To : Unknown

Continuation Sheet for CIOMS report

Dosage Text : 1) UNK

14). Drug : ZOFRAN [ONDANSETRON]
 Active Substance : 1) ONDANSETRON
 Form Strength :
 Form of Admin : 1) Unknown
 Route of Admin : 1) Unknown
 Indications : 1) Nausea [10028813 - Nausea]
 Therapy Dates : 1) From : 02/Aug/2022 To : Unknown
 Dosage Text : 1) UNK

23. OTHER RELEVANT HISTORY (Continuation...)

- 2) LEUKOPENIA (10024384 , Leukopenia) (01/Feb/2017 - Unknown) (Continuing : Unknown)
- 3) HYPERTENSION (10020772 , Hypertension) (11/May/2017 - Unknown) (Continuing : YES)
- 4) LARYNGEAL OPERATION (10062034 , Laryngeal operation) (12/Jun/2017 - Unknown) (Continuing : NO)
- 5) GASTROESOPHAGEAL REFLUX DISEASE (10017885 , Gastroesophageal reflux disease) (26/Jun/2017 - Unknown) (Continuing : Unknown)
- 6) COLONOSCOPY (10010007 , Colonoscopy) (15/Sep/2017 - Unknown) (Continuing : NO)
- 7) BIOPSY (10004720 , Biopsy) (15/Sep/2017 - Unknown) (Continuing : NO)
- 8) CHRONIC KIDNEY DISEASE (10064848 , Chronic kidney disease) (01/Jan/2018 - Unknown) (Continuing : Unknown)
- 9) PERIPHERAL SENSORY NEUROPATHY (10034620 , Peripheral sensory neuropathy) (01/Apr/2018 - Unknown) (Continuing : Unknown)
- 10) SIGMOIDOSCOPY (10040669 , Sigmoidoscopy) (02/Aug/2019 - Unknown) (Continuing : NO)
- 11) HEPATECTOMY (10061997 , Hepatectomy) (22/Aug/2019 - Unknown) (Continuing : NO)
- 12) MICROWAVE THERAPY (10068492 , Microwave therapy) (22/Aug/2019 - Unknown) (Continuing : NO)
- 13) CHOLECYSTECTOMY (10008611 , Cholecystectomy) (22/Aug/2019 - Unknown)
- 14) SEGMENTECTOMY (10042609 , Surgery) (07/Jul/2020 - Unknown) (Continuing : NO)
- 15) VENOUS RECONSTRUCTION (10080968 , Venous reconstruction) (07/Jul/2020 - Unknown) (Continuing : NO)
- 16) HERNIA DIAPHRAGMATIC REPAIR (10019913 , Hernia diaphragmatic repair) (07/Jul/2020 - Unknown) (Continuing : NO)
- 17) DRY SKIN (10013786 , Dry skin) (24/Apr/2023 - Unknown) (Continuing : Unknown)
- 18) SINUS TACHYCARDIA (10040752 , Sinus tachycardia) (11/Dec/2023 - Unknown) (Continuing : Unknown)
- 19) METASTATIC COLORECTAL CANCER (10052362 , Metastatic colorectal cancer) (Continuing : YES)

Study Information (Continuation...)

Study Name :

A Dose-Escalation and Expansion Study of the Safety and Efficacy of XL092 in Combination with Immuno-Oncology Agents in Subjects with Unresectable Advanced or Metastatic Solid Tumors

Past Therapy (ies)

Product Name : Oxaliplatin
 Indication : Colorectal cancer (10061451)
 Start Date : 03/Aug/2022
 Stop Date : 03/Apr/2023

Product Name : Oxaliplatin
 Indication : Colorectal cancer (10061451)
 Start Date : 05/Jun/2023
 Stop Date : 07/Aug/2023

Product Name : Oxaliplatin
 Indication : Colorectal cancer (10061451)
 Start Date : 25/Sep/2017
 Stop Date : 02/Apr/2018

Product Name : Fluorouracil 5
 Indication : Colorectal cancer (10061451)
 Start Date : 21/Oct/2020

Continuation Sheet for CIOMS report

Stop Date	:	16/Aug/2021
Product Name	:	Fluorouracil 5
Indication	:	Colorectal cancer (10061451)
Start Date	:	01/Mar/2019
Stop Date	:	22/Jul/2019
Product Name	:	Fluorouracil 5
Indication	:	Colorectal cancer (10061451)
Start Date	:	16/Apr/2018
Stop Date	:	04/Mar/2019
Product Name	:	Capecitabine
Indication	:	Colorectal cancer (10061451)
Start Date	:	22/Oct/2019
Stop Date	:	08/Jul/2020
Product Name	:	Capecitabine
Indication	:	Colorectal cancer (10061451)
Start Date	:	01/Feb/2022
Stop Date	:	01/Aug/2022
Product Name	:	Fluorouracil 5
Indication	:	Colorectal cancer (10061451)
Start Date	:	25/Sep/2017
Stop Date	:	02/Apr/2018
Product Name	:	Capecitabine
Indication	:	Colorectal cancer (10061451)
Start Date	:	03/Aug/2022
Stop Date	:	03/Apr/2023
Product Name	:	Capecitabine
Indication	:	Colorectal cancer (10061451)
Start Date	:	22/Aug/2023
Stop Date	:	29/Jan/2024
Product Name	:	Capecitabine
Indication	:	Colorectal cancer (10061451)
Start Date	:	05/Jun/2023
Stop Date	:	07/Aug/2023
Product Name	:	Irinotecan
Indication	:	Colorectal cancer (10061451)
Start Date	:	01/Mar/2019
Stop Date	:	22/Jul/2019
Product Name	:	Irinotecan
Indication	:	Colorectal cancer (10061451)
Start Date	:	21/Oct/2020
Stop Date	:	16/Aug/2021
Product Name	:	Panitumumab
Indication	:	Colorectal cancer (10061451)
Start Date	:	21/Oct/2020
Stop Date	:	16/Aug/2021
Product Name	:	Panitumumab
Indication	:	Colorectal cancer (10061451)
Start Date	:	01/Mar/2019
Stop Date	:	22/Jul/2019
Product Name	:	Bevacizumab
Indication	:	Colorectal cancer (10061451)
Start Date	:	25/Sep/2017
Stop Date	:	02/Apr/2018

Continuation Sheet for CIOMS report

Product Name : Bevacizumab
Indication : Colorectal cancer (10061451)
Start Date : 22/Aug/2023
Stop Date : 29/Jan/2024

Product Name : Bevacizumab
Indication : Colorectal cancer (10061451)
Start Date : 05/Jun/2023
Stop Date : 07/Aug/2023

Product Name : Bevacizumab
Indication : Colorectal cancer (10061451)
Start Date : 03/Aug/2022
Stop Date : 03/Apr/2023

Product Name : Leucovorin
Indication : Colorectal cancer (10061451)
Start Date : 21/Oct/2020
Stop Date : 16/Aug/2021

Product Name : Bevacizumab
Indication : Colorectal cancer (10061451)
Start Date : 16/Apr/2018
Stop Date : 04/Mar/2019

Product Name : Leucovorin
Indication : Colorectal cancer (10061451)
Start Date : 25/Sep/2017
Stop Date : 02/Apr/2018

Product Name : Leucovorin
Indication : Colorectal cancer (10061451)
Start Date : 01/Mar/2019
Stop Date : 22/Jul/2019

Primary Reporter:

